



Nursing Practice Guideline

Post-Operative Management

Reference #: NPG57

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	Nursing

Introduction

This guideline outlines the procedures and responsibilities of individuals and the health service organisation in relation to post-operative management for ward-based patients. It ensures the delivery of a uniform, integrated multidisciplinary approach with management that is consistent with best practice.

Risk Statement

Non-compliance with this guideline will:

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☒	Misconduct	☐
Breach SCGH Mission & Values	☒	Staff Safety	☒
Other:	☐		

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1. Collection of the Patient Post Procedure

1.1 Medical Requirements

Post-operative orders are documented by the surgical team in the post-operative instruction section on the operation report.

Ensure that a written medical prescription has been documented for any analgesic infusion, e.g. Regional or Patient Controlled Intravenous Analgesia.¹

With the exception of patients destined for the Intensive Care Unit post operatively, all patients requiring continuous oxygen therapy on discharge from Post Anaesthetic Care Unit (PACU) are to have an oxygen prescription written by the Anaesthetist.

1.2 Safety Information

The patient must have met Post Anaesthetic Care Unit (PACU) discharge criteria before being returned to the ward. ^{1, 23, 4}

If patient meets escalation criteria on the Observation and Response Chart (ORC) they must have appropriate Senior nurse review with documentation in patient notes.

If patient meets escalation criteria for medical review on the ORC 'Time Limited Post Procedure Modifications' can be implemented by the Registrar or Consultant of the procedural team. The 'Time Limited Post Procedure Modifications' table must be completed, including parameter range, escalation required along with instructions for when observations fall outside of the ranges.

In addition to the above criteria:

OPH site specific:

The patient is to stay in PACU for a minimum of 20 minutes once conscious and orientated after a general anaesthetic and after administration of final dose of a narcotic opioid drug.

SCGH site specific:

There is no minimum length of stay in PACU unless patients have had opioid analgesia or have an epidural insitu. Following commencement of an epidural infusion in Post Anaesthetic Care Unit (PACU), the patient is to remain in the PACU for a minimum of 30 minutes for observation.

For additional information refer to the following SCGOPHCG Nursing Practice Guidelines:

- Opioid and Ketamine Intravenous Analgesia
- Regional Analgesia (Acute Pain)

Oxygen and suction **MUST** be present and accompany **ALL** patient transfers from the PACU to the ward.

1.3 Nursing Management

Preparation of the Ward Bed Space

- Prior to collecting the patient, prepare the bed space for the patient's return:
 1. Ensure the bed space is tidy and free of clutter.
 2. Ensure oxygen and suction equipment is readily available and functional.
 3. Ensure all other relevant post-operative equipment specific to the surgery or procedure is available as required.
- When the patient is ready for return to the ward, the PACU staff will phone and request the ward nurse to collect the patient

Clinical Handover of the Patient from PACU Nurse to Ward Nurse

The PACU Discharge Criteria must be met prior to transfer to the ward. The PACU Discharge Criteria includes^{31, 2, 3, 4} ,:

Airway & Breathing: Patient's Airway must be clear with the ability to cough and a respiratory rate ≥ 8 -24 breaths per minute

Oxygenation: Oxygen saturations equal or $>94\%$ on room air, or as per instruction of Anaesthetist.

Consciousness: The patient must be rousable to voice and responding with a sedation score equal or <3 on the PACU record/documentation.

Where there is a pre-existing neurological deficit, the patient may be discharged when they have returned to pre-op cognitive levels, limb movement and pupils are equal and reactive

Temperature: Core temperature should be greater than 36°C prior to discharge. Patients who arrive in PACU with a temperature $<36^{\circ}\text{C}$ must have their temperature monitored and documented at 30-minute intervals until their core temperature reaches 36°C .

If a patient is deemed suitable for discharge with a core temperature $<36^{\circ}\text{C}$., active warming methods that apply heat to the patient externally, either superficially to the skin or to the core can be used until patient can maintain a temperature of 36°C . Examples of active warming devices include Bair huggers and in consultation with Anaesthetist

intravenous fluid warmers. Passive warming devices may also be used to assist active warming methods. These include warm blankets, space blankets and foil hats.

Analgesia: Patient's pain has been optimally relieved.

Bladder management: ALL patients who meet the following criteria should undergo a bladder scan before discharge from PACU:

- Patients who have been in theatre for greater than or equal to 60 minutes duration AND who have received greater than or equal to 1 litre of intra-operative fluid, **OR** Time since last urinary void exceeds 4 hours

Skin Integrity: All patients' pressure points checked and documented within 2 hours of arrival into PACU.

Procedural or Surgical Problems: The Surgeon/Anaesthetist has been informed of surgical issues relating to blood loss from wounds or drains or changes to neurovascular or neurological status

Invasive lines: Arterial lines must be removed (except for GHU/G52 HDU or ICU/CCU or high acuity areas)

- All patients who have a Central Venous Catheter inserted in Theatre or PACU must have a CXR completed in the PACU and the X-Ray should be reviewed and documented 'safe for use' by the Anaesthetist prior to the patient leaving the PACU.

Documentation: The final set of core observations taken in PACU will be documented on the Observation and Response ORC chart Patients who meet escalation criteria on the ORC chart must have appropriate Senior Nurse or Medical reviews documented as necessary prior to the patient leaving the PACU.

- If the patient requires **time limited post procedural modifications** prior to leaving the PACU these must be completed by the anaesthetist or interventionist responsible for the procedure and are valid for 4hrs only.
- Documentation must be correctly completed, labelled, photocopied, and given to the ward nurse. The last set of PACU observations should be entered onto the adult observation response chart
- All the above criteria will be discussed using the ISoBAR method during the clinical handover of the patient from the PACU nurse to the ward receiving nurse, including⁴:
 - **I Identification**
 - Ward nurse to PACU nurse introduction
 - Ward nurse to patient introduction
 - 3-point Patient Identification check
 - **S Situation** - Procedural Information
 - Procedure performed
 - Type of anaesthesia provided (General, local, regional, combined)
 - Intra-operative complications if any
 - Specific post-anaesthesia orders, including oxygen therapy and duration.
 - Specific post-operative orders as per Operation Report
 - **Observations and Haemodynamic status of the patient**
 - Patient's core observations whilst in theatre and PACU – review of the Anaesthesia Record and the PACU Record
 - Procedure specific observations required. (Neurological, neurovascular, etc.
 - Visualisation and check of wounds, drains, IDC, PVC etc.
 - Visualisation and check of Intravenous infusions insitu
 - Review of medications given in theatre and PACU
 - Review of Intravenous fluids given in theatre and PACU, ensure lines and fluids are correctly labelled.

- Skin Integrity checked and variations discussed and documented
- **B Background** – Relevant Past Medical History
 - Discuss drug allergies and reactions (if known)
 - Patient co-morbidities and past history as per Anaesthesia and Perioperative Record
 - Specific concerns, i.e., Patient aggressive pre-operatively; patient uses CPAP overnight
- **A Agreed Plan**
 - Review specific post-operative orders as per the Operation Report
 - Type and location of dressing/s – note any excessive ooze or reinforcement required
 - Type and location of drain/s – note level of suction required; note volume of drainage and ensure drain connections are secure
 - Oxygen prescription,
 - Ongoing fluid management, analgesia and postoperative nausea and vomiting management
- **R Read back**
 - PACU nurse to confirm with the ward nurse that all information and instructions received are clear and understood.
 - This is the time to seek clarification if required
- At the completion of the verbal handover, the PACU nurse will sign the PACU record (SCGH) or Patient Safety Checklist (OPH) as discharging the patient and the ward nurse will sign the PACU record indicating acceptance of the patient into their care.

Transportation of the Patient from PACU to the Ward

- Before the patient is transferred to the ward, oxygen and suction equipment will be placed on the back of the bed by the PACU staff.
- Once the ward nurse has received clinical handover and taken responsibility for the patient, the theatre Hospital Service Assistant (HSA) (SCGH) or Theatre Orderly CARPS (OPH) will be provided to transfer the patient back to the ward.
- While in transit, the escorting nurse will position themselves to ensure there is always direct visualisation of the patient.

2. Continuing Care of the Patient on the Ward

2.1. Medical Requirements

Post-operative orders are documented by the surgical team in the post-operative instruction section on the Operation Report

Ensure that a written medical prescription has been documented for any analgesic infusion, e.g. Regional or Patient Controlled Intravenous Analgesia.¹

2.2. Nursing Management

Post-Operative Observations

On return to the ward, the following patient care must be provided:

- Refer to post-operative orders as outlined in the Operation Report
- Monitoring and frequency of Core observations

Local Anaesthetic:

Core observations including, assessment of wound site, pain score and Neurovascular observations (as required), to be completed a minimum of once on return to ward, unless increased frequency requested in the Operation Report

Intravenous Sedation:

OPH (Day Procedure)- Assess and document

- Core observations on arrival to ward, and then if within accepted parameters, record every 30 minutes for 1 hour, then 1-hourly for 2-hours.

- Refer to Surgical Services Standard 'Discharge Criteria for day patients.

SCGH (Day Procedure): Assess and Document

- Core observations a minimum of half hourly for 2 hours, or part thereof, or as otherwise directed by Medical Officer.

General Anaesthetic:

Core observations to be assessed and documented on the ORC Chart (MR 410 871) a minimum of:

- Half hourly for 2 hours
- 1 hourly for 2 hours
- 2 hourly for 4 hours
- 4 hourly for 24 hours, or as otherwise directed by a Medical Officer.
- If the patient's core observations fall into an escalation category, the appropriate escalation process must be followed every time, and actions documented as per the ORC Chart
- Additional frequent observations are required for patients with Regional Analgesia and / or Intravenous Opioid and Ketamine Analgesia.
- Observe wound/s and drain/s with each set of core observations and report and document concerning variances.
- Perform additional procedure specific observations as required:
 - Neurological observations post neurosurgery.
 - Neurovascular observations of extremities post arterial grafts and other Vascular surgery, Orthopaedic surgery and/or application of Plaster of Paris (POP).
 - Check thrill and bruit post formation of Arterio-Venous Fistula
- Assess pain and nausea levels with each set of core observations and action appropriately.
- Unless contra-indicated, coughing, deep breathing, and leg movements to be commenced as soon as the patient can co-operate.
- Accurate fluid balance intake and output to be measured and documented. This includes, oral fluids, intravenous fluids, urine output and drain output.
- Consider notifying NOK that patient has returned to the ward.

Review of Risk Assessments

- Patient risk assessments must be reviewed, and documentation updated on return to the ward:
 - The patient's falls risk must be reassessed, and the Falls Risk Management Plan updated accordingly
 - The patient's pressure injury risk must be reassessed, and the Pressure Injury Risk and Skin Integrity Management Plan updated accordingly

Bladder Management

All non-catheterised patients are at risk of post-operative urinary retention.

- Ensure the patient has passed urine **within 4 hours** of their last preoperative void.

Hygiene Needs

- Provide post-operative hygiene care upon return from theatre and until the patient is able to perform this independently. This includes, but is not limited to:
 - Post-operative bed bath and replace hospital gown and linen as required
 - Mouth care
 - Hair care
 - Catheter and perineal care

Nutrition

- Commence diet and fluids as directed by post-operative orders
- If there was an abdominal / GI intervention, then feeding should be resumed when deemed safe by the procedural or operative teams.

Post-Operative Enteral Feeding (ICU specific)

ICU Patients who were receiving enteral feeds pre-operatively:

- If no abdominal / GI interventions were undertaken, resume enteral tube feeding at previous rate as soon as possible.
- If there was an abdominal / GI intervention, then feeding should be resumed as early as deemed safe by the procedural and ICU teams. A reduced rate may be used initially.

Related Documents

Legislation

- Health Services Act 2016
- Medicines and Poisons Act 2014, Medicines and Poisons Regulations 2016
- Work Health and Safety Act WA 2020

Policies:

- WA Department of Health Mandatory Policy:
 - High Risk Medication Policy MP0131/20
 - Clinical Handover Policy MP0095/18
 - Medication Chart Policy MP0078/19
 - State-wide Medicines Formulary Policy MP0077/18
- SCGOPHCG Hospital Policy:
 - High Risk medication Policy HP#261
 - Patient Identification and Procedure Matching – Communicating for Safety HP#48
 - Medicines Management HP#141
 - SCGOPHCG Clinical Handover-Communicating for Safety HP #232
 - HSA Patient Transfers requiring Nurse Escort
 - Recognising and Responding to Acute Deterioration HP#234
- Guidelines:
 - SCGOPHCG NPG Bladder Management and Urinary Catheterisation
 - SCGOPHCG NPG Opioid and Ketamine Intravenous Analgesia
 - SCGOPHCG NPG Regional Analgesia Acute Pain
 - SCGOPHCG CPG Oxygen Therapy

References

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4. Phillips N, Street M, Kent B, Haessler E & Caddedu M. Post- anaesthetic discharge scoring criteria: key findings from a systematic review. International Journal of Evidence-Based Healthcare 2013;11(4) : 275-284.[Level 1]
5. Sivapuram M. Patient warming systems: Effects of patient warming methods (active vs passive) in patients undergoing major surgery [Evidence Summary on the Internet]. Adelaide: Joanna Briggs Institute; 2022 [updated Dec 2021] Available from: The Joanna Briggs Institute EBP Database, JBI @ Ovid (Guideline) Level 1.

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